INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Gemcitabine 800 mg/m² · 28-day schedule

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
28 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00749
Version 3

Gemcitabine 800 mg/m² is given on Days 1, 8 and 15 of each 28-day cycle, for six to eight cycles unless progression or unacceptable toxicity changes the plan.

Treatment stages

01 **Gemcitabine 800 mg/m² · 28-day schedule**
28-day reference

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

		PAGE
01	Understanding your treatment	2
02	Your schedule, tests and appointments	4
03	Symptoms and supportive care	9
04	Your record and contact card	14
05	Sources and further information	16

My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

SACTCHECK · CHAPTER 01

Understanding your treatment

Gemcitabine 800 mg/m² · 28-day schedule · National Cancer Control Programme (NCCP) 00749

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Contact oncology promptly for fever, shivering, unusual bleeding, severe mouth or bowel symptoms, inability to drink, reduced urine, new breathlessness or a painful/leaking line. Severe chest pain, collapse or severe breathing difficulty needs emergency help.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

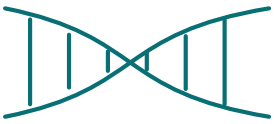
How the medicines work

Control advanced cancer and symptoms, with regular review of benefit and side effects.

ANTIMETABOLITE CHEMOTHERAPY

Gemcitabine

intravenous (IV) infusion



Interferes with deoxyribonucleic acid (the cell's genetic material) (DNA) synthesis

Gemcitabine is converted inside cells into forms that disrupt DNA building blocks and DNA copying. It can affect normal blood-cell production as well as cancer cells.

Activated inside cells → DNA production disrupted → Cell division slowed

What to watch for

Report fever, unusual bruising, breathlessness, swelling or reduced urine urgently. Flu-like symptoms cannot safely be assumed to be a harmless drug effect.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Why this treatment?

This gemcitabine schedule may be considered for advanced breast cancer after two or more previous lines of treatment. The aim is cancer and symptom control, balanced against side effects, previous treatment, cancer biology and your priorities.

Treatment setting

Advanced breast cancer after two or more treatment lines

Your exact schedule

Gemcitabine 800 mg/m² is given on Days 1, 8 and 15 of each 28-day cycle, for six to eight cycles unless progression or unacceptable toxicity changes the plan.

Alternatives

Discuss suitable endocrine, targeted or chemotherapy options, expected benefit, practical demands and the option of supportive care according to your setting.

Why this treatment may be chosen

Show trial results and what the numbers mean

- Pembrolizumab + chemotherapy
- Placebo + chemotherapy

Your team can discuss expected benefit, alternatives and uncertainty for your own cancer.

SACTCHECK · CHAPTER 02

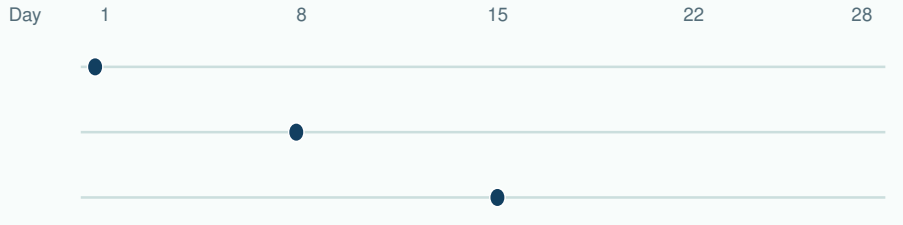
Your schedule, tests and appointments

Gemcitabine 800 mg/m² · 28-day schedule · NCCP 00749

The phases and how they connect

Gemcitabine 800 mg/m² · 28-day schedule · 28-day reference

28-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

28-DAY REFERENCE

Gemcitabine 800 mg/m² · 28-day schedule

28-day interval

Gemcitabine 800 mg/m² is given on Days 1, 8 and 15 of each 28-day cycle, for six to eight cycles unless progression or unacceptable toxicity changes the plan.

These are the medicines in this exact schedule. Dose holds, missed tablets and omitted visits need instructions from your treating team.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Relative milestones, not equal time intervals. Your team books tests and reviews results; this map does not confirm that either has happened.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00749 v3 · official regimen · NICE NG101 · bone health and follow-up · ASCO · biomarkers in metastatic breast cancer · ESMO metastatic breast cancer guideline · 2026

THE WHOLE TREATMENT JOURNEY

Gemcitabine 800 mg/m² · 28-day schedule: treatment and checks

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First treatment block	During treatment	Change / completion	Follow-up
Cancer medicines NCCP 00749	Confirm exact regimen	Gemcitabine 800 mg/m² · 28-day schedule	Gemcitabine 800 mg/m² is given on Days 1, 8 and 15 of each 28-day cycle, for six to eight cycles unless progression or unacceptable toxicity changes the plan.	Review active medicines and purpose	Next treatment / follow-up plan
Surgery / radiotherapy NCCP 00749	Advanced breast cancer after two or more treatment lines	Follow the multidisciplinary team (MDT) sequence	Radiotherapy timing needs agreement	Selected local treatment as indicated	Review recovery and ongoing effects
Blood / kidney / liver NCCP 00749	full blood count (FBC), renal and liver profile	FBC before Days 1, 8 and 15	Renal and liver tests before each cycle	Review unresolved abnormalities	Individual clinical plan
Heart / breathing NCCP 00749	Review symptoms and prior treatment	Investigate clinical concerns	No routine HER2-style echo schedule	Review new chest/ breathing symptoms	Clinical indication
Cancer review NCCP 00749 · METASTATIC · MARKERS	Confirm stage and setting	Agree review checkpoints	Review symptoms and response	Discuss continuation / next treatment	Setting-specific review
Other checks NCCP 00749 · BONE	Review previous treatment	Review support medicines	Review toxicity before each dose	Endocrine / bone plan where relevant	Follow unresolved symptoms

What each check is for

1

Baseline review

Before treatment

Advanced breast cancer after two or more treatment lines. There is no routine three-month echo schedule for CMF or gemcitabine alone. Review prior treatment and cardiac risk; new chest pain or breathing symptoms need urgent assessment.

Ask / act: Confirm source eligibility and results.

2 **Treatment-day check**

Before each planned dose

Full blood count before every planned treatment on Days 1, 8 and 15; kidney and liver tests before each cycle. Initial eligibility, new-cycle and later-day thresholds are different.

Ask / act: Confirm active medicines and whether a dose is held.

3 **Support plan**

At every phase or day change

Gemcitabine 800 mg/m² is given on Days 1, 8 and 15 of each 28-day cycle, for six to eight cycles unless progression or unacceptable toxicity changes the plan.

Ask / act: Choose the matching support script; do not merge phases.

4 **Response and follow-up**

Agreed review points

For metastatic disease, record the disease sites and agree the scan type and review interval before treatment. Review response, symptoms and toxicity together; worsening symptoms may need earlier assessment.

Ask / act: Review symptoms, results and patient priorities together.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your practical treatment plan

STEP 1

Before the first visit

Blood tests, medicines review and the consent discussion come first. Review previous treatment, organ function and the treatment-day blood-test plan.

STEP 2

Plan venous access

A cannula, peripherally inserted central catheter (PICC) or implanted port may be used. If a port is chosen, a trained team inserts it in a separate procedure with local anaesthetic and sometimes sedation; the team explains fasting, blood-thinner and wound instructions.

STEP 3

Infusion day

Staff check the line, give prescribed premedicines and treatment, and observe for reactions. Tell them immediately about burning, pain, rash or breathlessness.

STEP 4

Home and next review

This regimen does not routinely need a take-home infusion pump. Staff disconnect the infusion; a port is de-accessed when appropriate. Keep line-care and next-visit instructions.

Port insertion and removal

The line team explains placement, dressing care and when normal activities can resume. A port is accessed with a special needle during treatment. Removal of the implanted device is a separate planned procedure when no longer needed; do not remove or adjust it yourself.

PICC care and urgent line symptoms

A PICC usually needs regular dressing and flushing appointments even between treatments. Fever, redness, swelling, pain, leakage or damage requires prompt advice. Your hospital supplies the exact care instructions.

Visit time and transport

Allow time for blood review, preparation, treatment and observation. Your unit confirms the full visit duration. Arrange transport or an escort where advised.

Phase-specific visit pattern

Gemcitabine 800 mg/m² is given on Days 1, 8 and 15 of each 28-day cycle, for six to eight cycles unless progression or unacceptable toxicity changes the plan. No take-home infusion pump is part of this schedule. Port de-accessing after a visit is different from removal of the implanted port.

Checks across treatment

Your clinical team books tests, reviews results and explains any change. Contact them if an expected test or instruction is missing.

Blood and kidney tests

Full blood count before every planned treatment on Days 1, 8 and 15; kidney and liver tests before each cycle. Initial eligibility, new-cycle and later-day thresholds are different.

Heart and breathing

There is no routine three-month echo schedule for CMF or gemcitabine alone. Review prior treatment and cardiac risk; new chest pain or breathing symptoms need urgent assessment.

Cancer imaging

For metastatic disease, record the disease sites and agree the scan type and review interval before treatment. Review response, symptoms and toxicity together; worsening symptoms may need earlier assessment.

Bone health

Bone-density testing follows endocrine treatment, ovarian suppression, menopause and other bone-risk factors. dual-energy X-ray absorptiometry (bone density) (DEXA) is not automatically required before every chemotherapy course.

The wider treatment plan

For advanced cancer, review response and toxicity to decide whether to continue, change or stop. Supportive and palliative care can accompany anticancer treatment.

When treatment changes

Reconcile active cancer medicines, supportive medicines and remaining symptoms before every transition.

Follow-up

Keep the agreed review appointments and report new symptoms between visits; this map does not detect missed tests.

Hormone treatment and bone care

Hormone-receptor results and your treatment setting determine whether endocrine therapy is appropriate. It has its own monitoring plan.

Symptoms and supportive care

Gemcitabine 800 mg/m² · 28-day schedule · NCCP 00749

Gemcitabine 800 mg/m² · 28-day schedule · 28-day reference

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
IV treatment · Day 1	 **Day 1** Treatment in the unit
IV treatment · Day 8	 **Day 8** Treatment in the unit
IV treatment · Day 15	 **Day 15** Treatment in the unit
Blood counts / infection	 **Days 1–28** Fever needs urgent advice on any day
Fatigue	 **Days 1–28** May build across cycles
Nausea / appetite	 **Days 1–28** Follow the written prevention plan
Breathing / swelling	 **Days 1–28** New breathlessness or reduced urine: seek advice

General orientation: Symptoms can occur at any time, including days without a treatment visit. The graph does not predict symptom-free days.

Treatment days: A delayed or omitted dose needs an updated plan from the unit. Do not move doses yourself.

Why these bands stay broad: These broad symptom bands are educational possibilities, not measured risk curves. Infusion days are the planned schedule; delays require a new clinical plan. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00749 v3 · official regimen ↗

There are no guaranteed "safe days". Fever, shivers, sudden illness, severe breathing changes or chest pain need prompt assessment on any day.

Between-treatment notes

Day 1–28: One reference cycle

Gemcitabine 800 mg/m² is given on Days 1, 8 and 15 of each 28-day cycle, for six to eight cycles unless progression or unacceptable toxicity changes the plan. Use your written appointment and medicine plan; no dates are entered here.

Support around the medicines

1 **Anti-sickness plan**

Follow the prevention and rescue instructions prescribed for this schedule. Call if vomiting prevents drinking.

2 **Infection and bleeding**

Know the local urgent-contact number. Fever, shivering, sudden illness or unusual bleeding needs prompt advice even between visits.

3 **Medicine review**

Show the oncology pharmacist all prescribed, over-the-counter and herbal medicines. Ask before starting anything new.

4 **Hydration and symptoms**

Agree fluid intake with the team, especially with heart or kidney fluid restrictions. Report reduced urine, swelling, mouth ulcers or diarrhoea.

Questions worth asking

Who arranges the next tests?

Your treating service arranges and reviews tests. If instructions or appointments are missing, contact the team; the map does not book tests.

What if a dose is missed or I feel unwell?

Contact the unit for a revised plan. Do not double tablets, shift treatment dates or assume that an omitted dose should be made up.

Try explaining the plan back: Which medicines are due on which days? What symptoms mean you should call today? Which tests come next and who arranges them?

Gemcitabine

- Low blood counts, infection and bleeding
- Flu-like symptoms, fatigue, rash and swelling
- Uncommon serious lung or liver injury
- Rare haemolytic uraemic syndrome and kidney injury

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · Gemcitabine 800 mg/m² · 28-day schedule

Active medicines: Gemcitabine

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally once	Before chemotherapy on Day 1 Review QT risk, constipation, hepatic function and any alternative local regimen.
Metoclopramide	10 mg orally up to three times daily as needed	Rescue duration to be specified Review renal function, contraindications and interactions before prescribing.

Additional phase-specific support

- Confirm allergy/interaction review, blood tests and the source-specific renal/hepatic assessment before prescribing.
- Confirm the dose is going ahead before starting the linked support course. Reconcile rescue medicine duration rather than copying another treatment day.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Day 8 · Gemcitabine 800 mg/m² · 28-day schedule

Active medicines: Gemcitabine

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally once	Before chemotherapy on Day 8 Review QT risk, constipation, hepatic function and any alternative local regimen.
Metoclopramide	10 mg orally up to three times daily as needed	Rescue duration to be specified Review renal function, contraindications and interactions before prescribing.

Additional phase-specific support

- Confirm allergy/interaction review, blood tests and the source-specific renal/hepatic assessment before prescribing.

- Confirm the dose is going ahead before starting the linked support course. Reconcile rescue medicine duration rather than copying another treatment day.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Day 15 · Gemcitabine 800 mg/m² · 28-day schedule

Active medicines: Gemcitabine

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally once	Before chemotherapy on Day 15 Review QT risk, constipation, hepatic function and any alternative local regimen.
Metoclopramide	10 mg orally up to three times daily as needed	Rescue duration to be specified Review renal function, contraindications and interactions before prescribing.

Additional phase-specific support

- Confirm allergy/interaction review, blood tests and the source-specific renal/hepatic assessment before prescribing.
- Confirm the dose is going ahead before starting the linked support course. Reconcile rescue medicine duration rather than copying another treatment day.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00749 · VERSION 3

Gemcitabine 800 mg/m² · 28-day schedule

Gemcitabine 800 mg/m² is given on Days 1, 8 and 15 of each 28-day cycle, for six to eight cycles unless progression or unacceptable toxicity changes the plan.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Blood / infection

Gemcitabine

Report fever, unusual bruising or bleeding immediately.

2 Breathing

Gemcitabine

New cough or breathlessness needs prompt assessment.

3 Kidneys / urine

Gemcitabine

Report reduced urine, new swelling or sudden weakness.

4 Skin / swelling

Gemcitabine

Report a spreading rash, blistering or rapid swelling.

5 Nausea / appetite

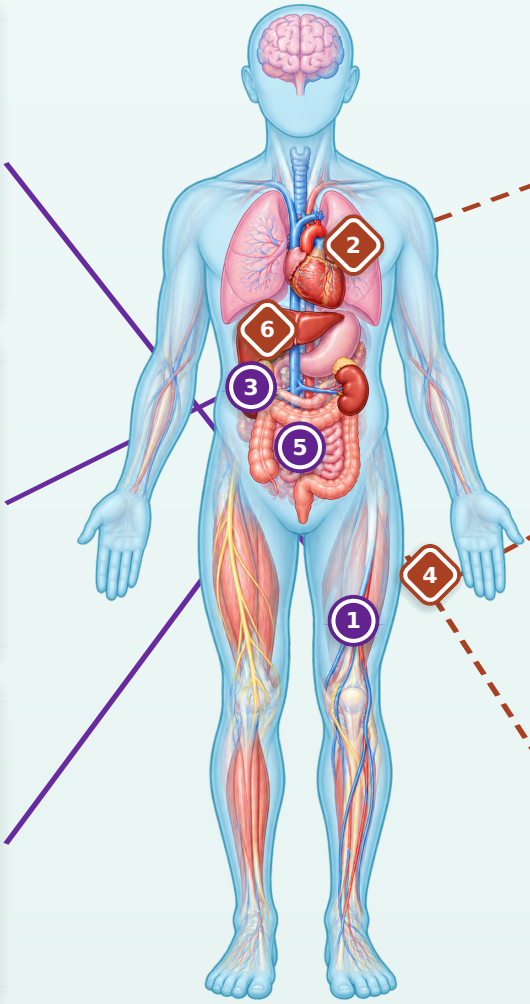
Chemotherapy

Call if vomiting prevents drinking.

6 Liver

Gemcitabine

Report yellow skin or eyes, dark urine or severe abdominal pain.



Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Medicine-specific effects are highlighted; this is not a complete list. New or persistent symptoms still need review after treatment stops.

Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

SACTCHECK · CHAPTER 04

Your record and contact card

Gemcitabine 800 mg/m² · 28-day schedule · National Cancer Control Programme (NCCP) 00749

Record each cycle separately. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Use a new record sheet for each cycle or review. Copy or print another sheet when needed; the number of sheets does not prescribe the length of treatment.

Cycle number: _____ Start date: _____ Next review: _____

Gemcitabine 800 mg/m² · 28-day schedule · reusable record · copy this sheet for another cycle of the same phase.

Gemcitabine · Antimetabolite chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999
Bleeding / bruising	□ None	□ New small bruise / brief bleed: report	□ Repeated bleeding: call today	□ Bleeding that will not stop: urgent help	□ Heavy bleeding / collapse: 112/999
Nausea	□ None	□ Queasy but eating and drinking	□ Intake reduced: contact the team	□ Unable to maintain intake: urgent review	□ Collapse / severe dehydration: 112/999

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Carry your treatment details

Print, cut out and fold. Follow the contact plan from your own hospital.

SACTCHECK · SACT TREATMENT

Gemcitabine 800 mg/m² · 28-day schedule

NCCP 00749 v3 · 28-day cycle

Gemcitabine 800 mg/m² is given on Days 1, 8 and 15 of each 28-day cycle, for six to eight cycles unless progression or unacceptable toxicity changes the plan.

1	2	3	4	5	6

Your contact plan

Oncology team: Insert numbers here
Out of hours: Insert numbers here.

Evenings / overnight: instructed out-of-hours pathway or nearest ED.
Emergency: 112 / 999.

Other hospital / own contact: _____

QR: https://sactcheck.com/patient/00749/ · no patient information.

Draft for review · SACTCheck v0.84.18

Sources and further information

Gemcitabine 800 mg/m² · 28-day schedule · National Cancer Control Programme (NCCP) 00749

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00749 v3 · official regimen**
   https://healthservice.hse.ie/documents/6672/749_v3_Gemcitabine_800_Monotherapy.pdf
2. **NICE NG101 · bone health and follow-up**
   https://www.nice.org.uk/guidance/ng101/chapter/Recommendations
3. **ASCO · biomarkers in metastatic breast cancer**
   https://ascopubs.org/doi/10.1200/JCO.2015.61.1459
4. **ESMO metastatic breast cancer guideline · 2026**
   https://www.annalsofoncology.org/article/S0923-7534(26)00902-6/fulltext
5. **eviQ · implanted port patient information**
   https://www.eviq.org.au/p/507
6. **eviQ · peripherally inserted central catheter (PICC) patient information**
   https://www.eviq.org.au/p/543
7. **ESMO 2024 early breast cancer guideline · treatment and follow-up**
   https://www.annalsofoncology.org/article/S0923-7534(23)05104-9/fulltext
8. **ESC 2022 cardio-oncology guideline · cardiac surveillance and risk**
   https://academic.oup.com/eurheartj/article/43/41/4229/6673995
9. **HSE · chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
10. **NOGG · section 7, aromatase inhibitors and fracture risk**
    https://www.nogg.org.uk/full-guideline/section-7-strategies-management-osteoporosis-and-fracture-risk
11. **NCCP NCIS antiemetics v8 (2025), Table 1**
    https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf